



Bill Of Supply

Name : Mrs. K MEGHANA
Age/Gender : 28 Y F
Contact No : +918332808860
Address : BANGLORE
UHID : RMAR.0000072295

Bill No : RMAR-OCS-763840
Bill/Reg Date : 01.12.2025 18:35
Referred by : Dr. GARIMA JAIN .
Center : Cradle Marathahalli
Emp No/Auth Code :

#	Department	Description Of Service	SAC/HSN Code	Qty	Rate	Gross Value	Discount	CGST Rate	CGST Amt	SGST/UTGST Rate	SGST/UTGST Amt	Net Value
1	Lab Tests	HIV RAPID	999316	1	748.00	748.00	0.00	0.00	0.00	0.00	0.00	748.00
2	Lab Tests	HBS AG SCREENING(RAPID)	999316	1	1,014.00	1,014.00	0.00	0.00	0.00	0.00	0.00	1,014.00
3	Lab Tests	VDRL (RAPID) BY ICT		1	481.00	481.00	0.00	0.00	0.00	0.00	0.00	481.00
4	Lab Tests	HCV Tri Dot	999316	1	1,079.00	1,079.00	0.00	0.00	0.00	0.00	0.00	1,079.00
5	Lab Tests	THYROID STIMULATING HORMONE (TSH), SERUM	999316	1	517.00	517.00	0.00	0.00	0.00	0.00	0.00	517.00
		TOTAL BETA- HCG										

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APOLLO SPECIALTY HOSPITALS PRIVATE LIMITED
 (Formerly known as Nova Specialty Hospitals Private Limited)
 CIN - U85100TG2009PTC099414
Regd. Office: 1-10-60/62, Ashoka Raghupathi Chambers, 5th Floor, Begumpet,
 Hyderabad - 500 016, Telangana

GSTIN: 29AADCN1803G1ZP

Address: Opus 143, 1st Cross, 5th Block,
 Koramangala, Bengaluru, Karnataka

6	Lab Tests	(TB-HCG)	999316	1	1,034.00	1,034.00	0.00	0.00	0.00	0.00	0.00	1,034.00
7	Package Charges	HEMOGRAM (CBP+ESR)	999312	1	630.00	630.00	0.00	0.00	0.00	0.00	0.00	630.00

Payments	Receipt No	Mode	Amount
Settlement			
01-12-2025	RMARRCA609550	Debit Card	5,503.00

Bill Amount:	5,503.00
Total Discount:	0.00
Patient Payment:	5,503.00
Patient Due:	0.00

Received with thanks: Five Thousand Five Hundred Three Rupees only

Authorized Signature :(Kamna Pariyar)

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Patient Name : Mrs.K MEGHANA
Age/Gender : 28 Y 0 M 15 D/F
UHID/MR No : RMAR.0000072295
Visit ID : RMAROPV764938
Ref Doctor : Dr.Dr GARIMA JAIN

Collected : 01/Dec/2025 06:37PM
Received : 01/Dec/2025 09:30PM
Reported : 01/Dec/2025 11:29PM
Status : Final Report

DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
TSH (Ultrasensitive/4thGen) , SERUM	4.090	µIU/mL	0.270-4.20	ECLIA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies. It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

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Dr.Nisha
M.B.B.S,MD(Pathology)
Consultant Pathologist



SIN No:SPL25047070

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Test Name	Result	Unit	Bio. Ref. Interval	Method
Total BETA- HCG (Tβ-HCG) , SERUM	5532	mIU/mL		ECLIA

Comment:

Category	hCG Levels (mIU/mL)
Non-pregnant premenopausal women	≤ 1
Healthy postmenopausal women	≤ 7

Weeks of Gestation Median (mIU/mL)	
5th-95th Percentile (mIU/mL)	
3 weeks	5.8 - 71.2
4 weeks	9.5 - 750
5 weeks	217 - 7138
6 weeks	158 - 31795
7 weeks	3697 - 163563
8 weeks	32065 - 149571
9 weeks	63803 - 151410
10 weeks	46509 - 186977
12 weeks	27832 - 210612
14 weeks	13950 - 62530
15 weeks	12039 - 70971
16 weeks	9040 - 56451
17 weeks	8175 - 55868
18 weeks	8099 - 58176

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DEPARTMENT OF SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBS AG SCREENING(RAPID) , SERUM	NON REACTIVE		NON REACTIVE	ICT

Comment:

Interpretation:

- This is a screening assay which detects the first serological marker of Hepatitis B as early as 4-16 weeks after exposure.
- It persists during acute illness and usually disappears 12-20 weeks after onset of symptoms. Persistence of HBsAg for more than 6 months indicates development of carrier state or chronic liver disease
- A negative test result does not exclude with certainty a possible exposure to or an infection with the hepatitis B virus.
- It is recommended that a positive result of HBsAg must be confirmed using a different enzyme immunoassay kit or by using a confirmatory assay based on neutralisation with human anti hepatitis B surface antibody and/or HBV PCR
- Based upon clinical history it may become necessary to test for presence of other markers of hepatitis B virus infection.

Test Name	Result	Unit	Bio. Ref. Interval	Method
HCV Tri Dot , SERUM	NEGATIVE		NEGATIVE	ICT

Comment:

The 4th Generation HCV TRI-DOT is a rapid, sensitive and qualitative in vitro diagnostic test for the detection of antibodies to Hepatitis C Virus in test specimen. It utilizes a unique combination of modified HCV antigens from the putative core, NS3, NS4 & NS5 regions of the virus to selectively identify all subtypes of Hepatitis C Virus in human serum/plasma with a high degree of sensitivity and specificity.

This is only a screening test and all reactive samples should be confirmed by HCV RNA PCR or confirmatory detection of HCV antibodies by serologic assays using recombinant HCV-specific antigens. The presence of anti-HCV does not imply a Hepatitis C infection but may be indicative of recent and / or past infection by HCV.

According to the present state of knowledge, it can be assumed that available assays for the detection of HCV antibodies cannot identify all infected blood samples or persons. A non-reactive result does not exclude the possibility of exposure to or infection with HCV.

Patients with auto-immune liver diseases, renal disorders may show falsely reactive results.

In case where there is lack of correlation between the test results and the clinical condition, the patient should be referred back to the laboratory at the earliest.

Test Name	Result	Unit	Bio. Ref. Interval	Method
HIV RAPID , SERUM	NON REACTIVE		NON REACTIVE	ICT

Comment:

This test is used for screening for HIV-1 and/or HIV-2 infection in asymptomatic patients. Negative HIV-1/-2 antigen and

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antibody screening test results usually indicate absence of HIV-1 and HIV-2 infection. Reactive HIV-1/-2 antibody screening test results suggest the presence of HIV-1 and/or HIV-2 infection, but it is not diagnostic for HIV infection. All samples detected reactive must be confirmed by using HIV Western Blot.

Some samples show cross reactivity for HIV antibodies. Following factors are found to cause false positive HIV antibody test results: Naturally occurring antibodies, Passive immunization, Leprosy, Renal Disorders, Tuberculosis, Myco-bacterium avium, Herpes simplex, Hypergammaglobulinemia, Malignant neoplasms, Rheumatoid arthritis, Tetanus vaccination, Autoimmune diseases, Blood Transfusion, Multiple myeloma, Haemophilia, Heat treated specimens, Lipemic serum, Anti-nuclear antibodies, T-cell leukocyte antigen antibodies, Epstein Barr virus, HLA antibodies and other retroviruses

Test Name	Result	Unit	Bio. Ref. Interval	Method
VDRL (RAPID) BY ICT , SERUM				
VDRL (Rapid) by ICT	NEGATIVE		Non-Reactive	

Comment:

Syphilis card(ICT) test is a rapid and immunochromatographic procedure for the qualitative detection of Treponemal antibodies. The reactive samples were tested with RPR(semiquantitative) flocculation method.

These are screening tests for syphilis. All reactive results should be confirmed with TPHA/FTA with clinical correlation.

***** End Of Report *****

Result/s to Follow:
HEMOGRAM

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6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only



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